A Plus Home Health Systems

OASIS

5670 W Flamingo Rd ste d
Las Vegas NV 89103
Phone: (702)720-7929 Fax: (702)342-8398
NPI: 1154824985

STAFF INFO
Name: Miller, Julie RN
Patient INFO
Name: Helms, James
DOB: 1935-08-30

OASIS Assessment: OASIS E2 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number:
297256

(M0014) Branch State:
Branch State: **Nevada**

(M0016) Branch ID Number:

(M0018) Attending Physician (NPI):
1477909786

Physician:
ROSSIDIS, MICHAEL

Physician Address:
7455 W WASHINGTON AVE STE 160 LAS VEGAS NV 89128
☐ Not Applicable

(M0020) Patient ID number:
178853

(M0030) Start of Care Date:
04/11/2026

(M0032) Resumption of Care Date:
mm/dd/yyyy
☑ Not Applicable

(M0040) Patient Name:

First Name:	Middle Name:	Last Name:	Suffix:
James		Helms	

(M0066) Birth Date:
08/30/1935

(A0810) Sex:
◉ 1. Male ○ 2. Female

(M0050) Patient State of Residence:
State of Residence: **Nevada**

(M0060) Patient Zip Code:
89002

(M0063) Medicare Number:
4EH4MV8XC96
☐ Not Applicable

(M0064) Social Security Number:
161307862
☐ Not Applicable

(M0065) Medicaid Number:
☑ Not Applicable

(M0080) Discipline Completing Assessment:
◉ 1. RN ○ 2. PT ○ 3. SLP/ST ○ 4. OT
Assigned Staff:
Miller, Julie RN

(M0090) Date Assessment Completed:
04/11/2026

(M0100) Assessment Reason:
◉ 1. Start of Care

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply:

- ☑ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☑ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ Z. None of the above

(A1110) Language:

A. What is your preferred language?
English

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
◉ 0. No ○ 1.Yes ○ 9. Unable to determine

(B1300) Health Literacy:
How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?
○ 0. Never ○ 1. Rarely ◉ 2. Sometimes ○ 3. Often ○ 4. Always ○ 7. Patient declines to respond ○ 8. Patient unable to respond

(M0102) Date of Physician-ordered Start of Care (Resumption of Care):
If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1255, Transportation, if date entered:
04/11/2026
☐ NA - No specific SOC/ROC date ordered by physician

(M0104) Date of Referral:
Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA:
mm/dd/yyyy

(M0150) Current Payment Sources for Home Care:
(Mark all that apply)

- ☐ 0. None; no charge for current services
- ☑ 1. Medicare (Traditional fee-for-service)
- ☐ 2. Medicare (HMO/managed care/Advantage plan)
- ☐ 3. Medicaid (Traditional fee-for-service)
- ☐ 6. Title programs (for example, Title III, V, or XX)
- ☐ 7. Other government (for example, TriCare, VA)
- ☐ 8. Private insurance
- ☐ 9. Private HMO/managed care

- [ ] 4. Medicaid (HMO/managed care)
- [ ] 5. Worker's compensation
- [ ] 10. Self-pay
- [ ] 11. Other (Specify):
- [ ] UK - Unknown

(A1255) Transportation (NACHC©):

In the past 12 months, has lack of reliable transportation kept you from medical appointments, meetings, work or from getting things needed for daily living?

- [x] 0. Yes
- [ ] 1. No
- [ ] 7. Patient declines to respond
- [ ] 8. Patient unable to respond

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(M1000) Inpatient Facilities that patient was discharged within the past 14 days?

(Mark all that apply)

- [ ] 1. Long-term nursing facility (NF)
- [ ] 2. Skilled nursing facility (SNF/TCU)
- [ ] 3. Short-stay acute hospital (IPPS)
- [ ] 4. Long-term care hospital (LTCH)
- [ ] 5. Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6. Psychiatric hospital or unit
- [ ] 7. Other (specify):
- [x] NA - Patient was not discharged from an inpatient facility

(M1005) Inpatient Discharge Date (Most Recent):

mm/dd/yyyy [ ] UK - Unknown

Primary Reason for Home Health/Medical Necessity (POC Element):

James Helms, a 90-year-old male, is being referred for home health care following an encounter for change or removal of a non-surgical wound dressing, with a chronic ulcer on another part of the left foot with the fat layer exposed; he recently underwent a revascularization procedure due to significant Peripheral Artery Disease and has a left second toe ulceration at risk for infection requiring skilled wound care, with additional conditions including cellulitis of the left foot and a blister on the left foot, and following vascular surgery he continues to experience pain and significant swelling in his lower extremities, indicating compromised circulation and the need for ongoing monitoring and skilled nursing care.

James's medical status reveals that he is alert and oriented but faces functional limitations due to pain and swelling, which hinders his mobility and daily activities. He requires skilled nursing care for the management of his complex wound care regimen, including dressing changes that need to occur every other day, as well as education on recognizing signs of infection. Given his age and the potential risks associated with his conditions, the skilled services provided by home health, including nursing, wound care, and possibly physical therapy, are medically necessary to facilitate his recovery and prevent further complications.

Additionally, James's recent discharge from a vascular procedure underscores the urgency of home health admission, as his clinical needs must be addressed in a timely manner to promote healing. The comprehensive care plan will focus on wound management, infection prevention, and enhancing his overall health status through skilled interventions, ensuring that he receives the appropriate care in the comfort of his home. Therefore, the combination of his medical history, functional limitations, and the skilled care required to manage his conditions justify the need for home health care services.

Homebound Reason (POC Element):

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [x] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [ ] Cane
  - [ ] Manual Wheel Chair
  - [x] Walker
  - [ ] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [ ] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Patient requires assistance for safety with ADLs/IADLs.

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadprilegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [x] Experiencing pain impairs/restricts mobility
- [x] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [ ] Leaving home is exhausting, extended rest periods to recover
- [ ] Extended respiratory recovery after exertion
- [x] Requires human assist due to unsteady gait/poor balance
- [ ] Weakness and pain from recent surgery
- [x] Visually impaired-requires constant attendance/assist
- [ ] Cognitively impaired, unable to safely leave home without assist
- [x] Infectious disease process causing weakness and exhaustion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [ ] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [ ] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [x] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [x] Activities restricted due to pain limitations

Other Homebound Reason :

High risk for fall and injury

Prognosis (POC Element (§484.60 (2.v))):

○ 1-Poor ○ 2-Guarded ● 3-Fair ○ 4-Good ○ 5-Excellent

Functional Limitations (POC Element (§484.60 (2.vi))):

- ○ 1 - Amputation
- ○ 2 - Bowel/Bladder (Incontinence)
- ○ 3 - Contracture
- ○ B- Other (Specify)
- ● 4 - Hearing
- ○ 5 - Paralysis
- ● 6 - Endurance
- ● 7 - Ambulation
- ○ 8 - Speech
- ○ 9 - Legally blind
- ○ A - Dyspnea with minimal exertion

Emergency Preparedness (POC Element):

Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
Notified local utility companies and local emergency offices notified of life supporting equipment being used
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
Educational materials provided to suggest/assist with emergency management/decision making priorities
List of local and state approved evacuation routes and community shelters relevant to the patient's specific geographic location
Written materials to restate/reinforce the emergency preparedness procedures given to the Patient.
Discussed patient specific emergency planning options
Obtained the patient's emergency contact numbers for the medical record
List of recommended items to have prepared/ready and available in the event of an emergency

Acuity Level:
Acuity Level: **Medium Risk or Medium Priority**

Emergency Triage:
Emergency Triage: **Category 2 - Not life threatening but would suffer severe adverse effect**

Additional Emergency Preparedness Information:

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments:

Advance Directives (POC Element (§484.60 (2.xv))):

Patient has Advance Directives? ○ Yes ○ No

If you have checked "yes", fill the following fields:

- [ ] Healthcare Representative
- [ ] Do Not Resuscitate
- [x] Power of Attorney
- [x] Living Will
- [ ] Conservator Assigned
- [ ] Organ Donor
- [ ] Funeral Arrangements Made
- [ ] Full Code
- [ ] The patient has not executed any advance directives to health care.
- [x] The patient was provided with written and verbal information on advance directives.

Comments:
Power of Attorney Additional Info:
Michelle Helms-daughter 6027637484

Has surrogate? ● Yes ○ No

Surrogate Name: Michelle Helms

Surrogate Phone: 6027637484

Health Screening:

Last Cholesterol Level: mm/dd/yyyy

Last Mammogram: mm/dd/yyyy

Does patient perform monthly self breast exam? ○ Yes ○ No

Last Pap Smear: mm/dd/yyyy

Last PSA: mm/dd/yyyy

Last Prostate Exam: mm/dd/yyyy

Last Colonoscopy: mm/dd/yyyy

ACTIVE DIAGNOSES

(M1021/1023) Diagnoses and Symptom Control (POC Element (§484.60 (2.i))):

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

	Onset Date	Severity	
PRIMARY DIAGNOSIS Z48.00 - Encounter for change or removal of nonsurg wound dressing	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate
OTHER DIAGNOSIS - 1 I73.9 - Peripheral vascular disease, unspecified	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate
OTHER DIAGNOSIS - 2 L97.522 - Non-prs chronic ulcer oth prt left foot w fat layer exposed	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate
OTHER DIAGNOSIS - 3 S90.822D - Blister (nonthermal), left foot, subsequent encounter	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate
OTHER DIAGNOSIS - 4 L03.116 - Cellulitis of left lower limb	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate
OTHER DIAGNOSIS - 5 Z48.812 - Encntr for surgical aftcr following surgery on the circ sys	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate
Z98.62 - Peripheral vascular angioplasty status	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate
Z91.81 - History of falling	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate
Z79.01 - Long term (current) use of anticoagulants	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate
Z79.82 - Long term (current) use of aspirin	04/11/2026	0 1 2 3 4 ^	Onset Exacerbate

(M1028) Active Diagnoses:

Comorbidities and Co-existing Conditions *– Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.*

- [x] **1. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)**
- [ ] **2. Diabetes Mellitus (DM)**
- [ ] **3. None of the above**

VITAL SIGNS & PAIN ASSESSMENT

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up:

A. Height (in inches). Record most recent height measure since the most recent SOC/ROC:
68

B. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.:
145

BMI:
22.04

Comments:

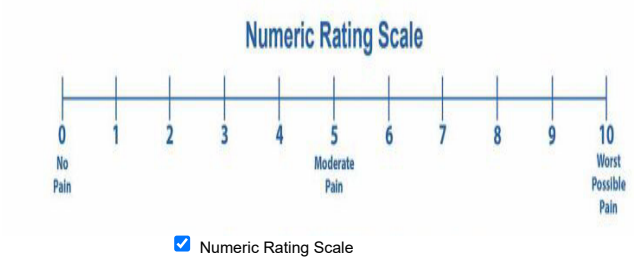
Vital Signs

Temperature: 97.6 Temporal
Respiratory (per minute): 18
Blood Pressure mm/Hg (R): 120 / 62 sitting
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 94
Comments:

Pulse (per minute): Apical = Radial = 83
Weight: 145 actual
Blood Pressure mm/Hg (L): /

O2 Sat in: Room Air

Pain Assessment



5

Collected Using: [ ] Face Scale [x] Numeric Rating Scale

Pain Location and Description:

1 **Location**
left dorsal foot

Start Date
04/11/2026

Activity
At Rest×

What makes pain worse?
Movement×

Intensity
5×

Duration
months

What makes pain better?
Rest×

Does pain affect quality of life?
yes

Unable to Assess [ ]

Description
Sharp×

Frequency
Frequent×

Unable to Assess [ ]

Type of Intervention
Non Phamacologic×

How well it is working
Fairly Well×

Status
COC×

Comments:

(J0510) Pain Effect on Sleep:

Ask patient: *"Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"*

- ( ) **0. Does not apply – I have not had any pain or hurting in the past 5 days**
- ( ) **1. Rarely or not at all**
- (x) **2. Occasionally**
- ( ) **3. Frequently**
- ( ) **4. Almost constantly**
- ( ) **8. Unable to answer**

(J0520) Pain Interference with Therapy Activities:

Ask patient: *"Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"*

- (x) **0. Does not apply – I have not received rehabilitation therapy in the past 5 days**
- ( ) **1. Rarely or not at all**
- ( ) **2. Occasionally**
- ( ) **3. Frequently**
- ( ) **4. Almost constantly**
- ( ) **8. Unable to answer**

(J0530) Pain Interference with Day-to-Day Activities:

Ask patient: *"Over the past 5 days, how often you have limited your day to day activities (excluding rehabilitation therapy session) because of pain?"*

( ) **1. Rarely or not at all** ( ) **2. Occasionally** (x) **3. Frequently** ( ) **4. Almost Constantly** ( ) **8. Unable to answer**

MEDICATION & ALLERGIES (INJECTABLE MEDICATIONS)

Medication (POC Element (§484.60 (2.x))):

	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	04/11/2026	NORVASC	2.5 mg 1 Tab Once a day	By mouth	CALCIUM CHANNEL BLOCKERS /	New
☐	04/11/2026	Atorvastatin (Oral Pill)	80 mg 1 Tab Once a day	By mouth	ANTILIPEMIC AGENTS /	New
☐	04/11/2026	Finasteride (Oral Pill)	5 mg 1 Tab Once a day	By mouth	HORMONES/SYNTHETICS/MODIFIERS,OTHER /	New
☐	04/11/2026	Lisinopril (Oral Pill)	40 mg 1 Tab Once a day	By mouth	ACE INHIBITORS /	New
☐	04/11/2026	Metoprolol (Oral Pill)	25 mg 1 Tab Once a day	By mouth	BETA BLOCKERS/RELATED /	New
☐	04/11/2026	Warfarin (Oral Pill)	2.5 mg 1 Tab Once a day	By mouth	ANTICOAGULANTS /	New
☐	04/11/2026	Warfarin (Oral Pill)	5 mg 1 Tab Once a day	By mouth	ANTICOAGULANTS /	New
☐	04/11/2026	ASPIR-LOW	81 mg DR Tab Once a day	By mouth	Salicylate /	New

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

Allergies (POC Element):

Name	Reaction	Start Date
		None known

☐ Add Allergies

(M2001) Drug Regimen Review:

Did a complete drug regimen review identify potential clinically significant medication issues?

- ● **0. No – No issues found during review [Go to M2010, Patient/Caregiver High-Risk Drug Education]**
- ○ **1. Yes – Issues found during review**
- ○ **Not assessed/no information**
- ○ **9. NA – Patient is not taking any medications [Go to O0110, Special Treatments, Procedures, and Programs]**

(M2003) Medication Follow-up:

Did the agency contact a physician (or physician designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

○ **0. No** ○ **1. Yes** ○ **Not assessed/no information**

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking 1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication Noted 2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	■
E. Anticoagulant	☑	☑
F. Antibiotic	☐	■
H. Opioid	☐	■
I. Antiplatelet	☑	☑
J. Hypoglycemic (including insulin)	☐	■
Z. None of the Above	☐	

(M2010) Patient/Caregiver High-Risk Drug Education:

Has the patient/caregiver received instruction on special precautions for all high risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ○ **0. No**
- ● **1. Yes**
- ○ **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

(M2020) Management of Oral Medications:

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. ***(NOTE: This refers to ability, not compliance or willingness.)***

- ○ **0. Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ● **1. Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ○ **2. Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ○ **3. Unable to take medication unless administered by another person**
- ○ **NA - No oral medications prescribed**

(M2030) Management of Injectable Medications:

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ◯ 0. Able to independently take the correct medication(s) and proper dosage(s) at the correct times
- ◯ 1. Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.
- ◯ 2. Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection
- ◯ 3. Unable to take injectable medication unless administered by another person
- ☑ NA - No injectable medications prescribed.

(O0110) Special Treatments, Procedures, and Programs:

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	☐
A3. Oral	☐
A10. Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐
C2. Continuous	☐
C3. Intermittent	☐
C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐
G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐
H10. Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

Most Recent Immunizations:

Pneumonia:	Flu:	Tetanus:	TB:
☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy
TB Exposure:	**Hepatitis B:**	**COVID-19:**	**Additional Immunization:**
☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy

Comments:
SN taught patient/PCG on the risks and benefits of up-to-date immunization.

NEUROLOGICAL (HEAD, MOOD, EYES, EARS)

Neuro/Emotional/Behavioral Assessment

Neuromuscular:

☐ No Problems Identified

Level of Consciousness:

☑ Awake ☑ Alert ☐ Lethargic ☐ Minimally Responsive ☐ Unresponsive ☐ Coma

Impairment:

☐ Speech ☑ Hearing ☑ Visual ☐ Tremors ☑ Numbness ☑ Tingling ☐ Vertigo ☐ Ataxia ☐ Decreased Sensitivity

Speech Impairment (Specify):

Psychosocial (POC Element (§484.60 (2.ii))):

☐ **Document as Clinical Narrative**

Educational level

Primary Language

English

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language

◯ **Needs interpreter**

Learning

◯ **Mental**
◯ **Physical**
◯ **Psychosocial**
◯ **Functional**

Emotions Reported:

◯ **Angry**
◯ **Discouraged**
◯ **Withdrawn**
◯ **Flat affect**
◯ **Difficulty coping**
◯ **Disorganized**

◯ **Depressed** ◯ **Recent** ◯ **Long Term**

Treatment:

Evidence of abuse/neglect/exploitation:

◯ **Potential**
◯ **Verbal/Emotional**
◯ **Physical**
◯ **Actual**
◯ **Financial**
◯ **No Evidence of Abuse/Neglect/Exploitation**

Unable to cope with altered health status, patient:

◯ **Lacks motivation** ◯ **Has unrealistic expectations** ◯ **Denies problems** ◯ **Unable to recognize problems**

Sleep/Rest

● **Adequate** ◯ **Inadequate**

No. of hours slept per night: **6**

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician

☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☐Yes ☑No ☐NA

Did the patient have a job before admission? ◯Yes ◉No

If yes, do they want to drive post-discharge? ☐Yes ☐No

If yes, do they want to return to work post-discharge? ◯Yes ◉No

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ◯ **No (patient is rarely/never understood)** ● **Yes** ◯ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◯ **None** ◯ **One** ◯ **Two** ● **Three** ◯ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year

◯ **0. Missed by > 5 years or no answer**
◯ **1. Missed by 2-5 years**
◯ **2. Missed by 1 year**
● **3. Correct**
◯ **Not assessed/no information**

Ask patient: **"What month are we in right now?"**

B) Able to report correct month

◯ **0. Missed by > 1 month or no answer**
◯ **1. Missed by 6 days to 1 month**
● **2. Accurate within 5 days**
◯ **Not assessed/no information**

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week

◯ **0. Incorrect or no answer**
● **1. Correct**
◯ **Not assessed/no information**

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"

◯ **0. No – could not recall**
◯ **1. Yes, after cueing ("something to wear")**
● **2. Yes, no cue required**
◯ **Not assessed/no information**

B) Able to recall "blue"

◯ **0. No – could not recall**
◯ **1. Yes, after cueing ("a color")**
● **2. Yes, no cue required**
◯ **Not assessed/no information**

C) Able to recall "bed"

◯ **0. No – could not recall**
◯ **1. Yes, after cueing ("a piece of furniture")**
● **2. Yes, no cue required**
◯ **Not assessed/no information**

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** **15**

(C1310) Signs and Symptoms of Delirium (from CAM©):

A. Acute Onset of Mental Status Change:

Is there evidence of an acute change in mental status from the patient's baseline?

● **0. No** ◯ **1. Yes**

B. Inattention:

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- [x] **0. Behavior not present**
- [ ] **1. Behavior continuously present, does not fluctuate**
- [ ] **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- [ ] **Not assessed/no information**

C. Disorganized thinking:

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- [x] **0. Behavior not present**
- [ ] **1. Behavior continuously present, does not fluctuate**
- [ ] **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- [ ] **Not assessed/no information**

D. Altered level of consciousness:

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- ***vigilant*** *– startled easily to any sound or touch lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch*
- ***stuporous*** *– very difficult to arouse and keep aroused for the interview*
- ***comatose*** *– could not be arouse*

- [x] **0. Behavior not present**
- [ ] **1. Behavior continuously present, does not fluctuate**
- [ ] **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- [ ] **Not assessed/no information**

(D0150) Patient Mood Interview (PHQ-2 to 9):

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	No ×	Never or 1 day
B. Feeling down, depressed, or hopeless	No ×	Never or 1 day
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		0

(D0700) Social Isolation:

How often do you feel lonely or isolated from those around you?

- [ ] **0. Never**
- [x] **1. Rarely**
- [ ] **2. Sometimes**
- [ ] **3. Often**
- [ ] **4. Always**
- [ ] **7. Patient declines to respond**
- [ ] **8. Patient unable to respond**

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed):

(Mark all that apply)

- [ ] **1. Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required**
- [ ] **2. Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions**
- [ ] **3. Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.**
- [ ] **4. Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)**
- [ ] **5. Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)**
- [ ] **6. Delusional, hallucinatory, or paranoid behavior**
- [x] **7. None of the above behaviors demonstrated**

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- [x] **0. Never**
- [ ] **1. Less than once a month**
- [ ] **2. Once a month**
- [ ] **3. Several times each month**
- [ ] **4. Several times a week**
- [ ] **5. At least daily**

(M1700) Cognitive Functioning:

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- [x] **0. Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.**
- [ ] **1. Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.**
- [ ] **2. Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility**
- [ ] **3. Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.**
- [ ] **4. Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.**

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- [x] **0. Never**
- [ ] **1. In new or complex situations only**
- [ ] **2. On awakening or at night only**
- [ ] **3. During the day and evening, but not constantly**
- [ ] **4. Constantly**
- [ ] **5. Patient nonresponsive**

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- [ ] **0. None of the time**
- [x] **1. Less often than daily**
- [ ] **2. Daily, but not constantly**
- [ ] **3. All of the time**
- [ ] **NA - Patient nonresponsive**

Mental Status (POC Element (§484.60 (2.ii)):

- [x] **1 - Oriented**
- [ ] **2 - Comatose**
- [ ] **3 - Forgetful**

- ◯ 4 - Depressed
- ◯ 7 - Agitated
- ◯ 10 - Dementia
- ◯ 13 - Wanders
- ◯ 5 - Disoriented
- ◉ 8 - Alert
- ◯ 11 - Assaultive
- 14 - Other
- ◯ 6 - Lethargic
- ◯ 9 - Confused
- ◯ 12 - Delirious

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Has there been a sudden/acute change in their mental status? ☐ Yes ☑ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☐ No ☐ Yes

Comments:

Cultural:

Is religion important to the patient? ◯ Yes ◉ No

Patient's primary source of emotional support:

Eyes and Ears

(B1000) Vision:

Ability to see in adequate light *(with glasses or other visual appliances)*

- ◯ 0. Adequate – sees fine detail, such as regular print in newspapers/books
- ◉ 1. Impaired – sees large print, but not regular print in newspapers/books
- ◯ 2. Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- ◯ 3. Highly impaired – object identification in question, but eyes appear to follow objects
- ◯ 4. Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Vision Assessment:

- ☐ No Problems Identified
- ☑ Glasses
- ☐ Contact Lenses: ☐ Right ☐ Left
- ☐ Glaucoma: ☐ Right ☐ Left
- ☐ Blurred Vision: ☐ Right ☐ Left
- ☐ Macular Degeneration: ☐ Right ☐ Left
- ☐ Blindness: ☐ Right ☐ Left
- ☐ Cataracts: ☐ Right ☐ Left
- ☐ Other (Specify):

Pupils:

- ☑ PERRLA
- ☐ Anisocoria: ☐ Right ☐ Left
- ☐ Other (Specify):

Comments:

(B0200) Hearing:

Ability to hear *(with hearing aid or hearing appliances if normally used)*

- ◯ 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV
- ◯ 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- ◉ 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- ◯ 3. Highly impaired – absence of useful hearing

Hearing Assessment:

- ☐ No Problems Identified
- ☑ Right Hearing Impairment
- ☑ Left Hearing Impairment
- ☐ Deaf
- ☐ Hearing Aids: ☐ Right ☐ Left

Comments:

CARDIOPULMONARY (CHEST & THORAX)

Heart Sounds:

- ☑ No Problems Identified
- ☐ Irregular: ☐ Known Hx ☐ New Finding
- ☐ Gallops: ☐ Known Hx ☐ New Finding
- ☐ Clicks: ☐ Known Hx ☐ New Finding
- ☐ Murmur: ☐ Known Hx ☐ New Finding
- ☐ FrictionRub: ☐ Known Hx ☐ New Finding

Comments:

Edema:

- ☐ Rigth Upper Extremities:
- ☐ Left Upper Extremities:
- ☑ Rigth Lower Extremities: Pitting, +2
- ☑ Left Lower Extremities: Pitting, +2

Others:

Others:

Comments:

Peripheral Pulses:

Radial: ☑ Strong ☐ Bounding ☐ Weak ☐ Thready ☐ Absent

Pedal: ☑ Strong ☐ Bounding ☐ Weak ☐ Thready ☐ Absent

Fluid Imbalance and Systemic Distress:

- ☐ Cramps
- ☐ Claudication
- ☐ Fatigue
- ☐ Face
- ☐ Neck
- ☐ Abdomen

Capillary Refill:

- ☑ < 3 secs
- ☐ > 3 secs

Chest/Thorax

Chest Pain:

- ☑ Denies
- ☐ Anginal
- ☐ Postural
- ☐ Localized
- ☐ Substernal
- ☐ Sharp
- ☐ Dull
- ☐ Aching
- ☐ Radiating
- ☐ Viselike
- ☐ Stabbing
- ☐ Nausea

☐ Other (Specify):

Associated with:

☐ Shortness of Breath ☐ Activity ☐ Rest ☐ Diaphoresis ☐ Frequency/Duration:

(M1400) When is the patient dyspneic or noticeably Short of Breath?

- ☑ 0. Patient is not short of breath
- ○ 1. When walking more than 20 feet, climbing stairs
- ○ 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- ○ 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- ○ 4. At rest (during day or night)

Respiratory Status:

Respiratory Distress Indicators:

☐ Dyspnea: ☐ At Rest ☐ With Exertion/Activity ☐ Accessory Muscle Used ☐ Orthopnea ☐ Retraction ☐ Stridor

☐ O2 Liters per minute: ☐ O2 Saturation (%):

Frequency/Timing of Oxygen Use:

☐ PRN ☐ Continuous ☐ Only at bedtime ☐ Other (Specify):

Route/Delivery:

☐ Nasal Cannula ☐ Trach Cuff ☐ Mask (Specify): ☐ Other (Specify):

Lung Sounds:

Anterior Right:	**Anterior Left:**		
Anterior Right: **Clear**	Anterior Left: **Clear**		
Posterior Upper Right:	**Posterior Upper Left:**	**Posterior Lower Right:**	**Posterior Lower Left:**
Posterior Upper Right: **Clear**	Posterior Upper Left: **Clear**	Posterior Lower Right: **Diminished**	Posterior Lower Left: **Diminished**

Comments:

Cough:

Cough Characteristics:

☐ None ☑ Dry ☐ Acute ☐ Chronic

Productivity of Cough:

☑ Non-Productive

☐ Productive:

Sputum Characteristics (if Productive):

☐ Brown ☐ Clear ☐ Cloudy ☐ Bloody ☐ Yellow ☐ White

☐ Hemoptysis: ☐ Amount (Specify): ☐ Frequency (Specify):

GASTROINTESTINAL & GENITOURINARY

Gastrointestinal (Mouth - Anus)

Mouth and Throat (Assess oral health and swallowing ability):

☐ Dentures: ☐ Upper ☐ Lower ☐ Partial

☐ Bleeding Gums/Gingivitis ☐ Toothache ☐ Ulcerations ☐ Lesion

☐ Dysphagia ☐ Hoarseness ☐ Sore Throat ☐ Masses/Tumors

Comments:
missing teeth

(M1870) Feeding or Eating:

*Current ability to feed self meals and snacks safely. Note: This refers only to the process of **eating**, **chewing**, and **swallowing**, not preparing the food to be eaten.*

- ○ 0. Able to independently feed self.
- ☑ 1. Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.
- ○ 2. Unable to feed self and must be assisted or supervised throughout the meal/snack.
- ○ 3. Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.
- ○ 4. Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.
- ○ 5. Unable to take in nutrients orally or by tube feeding.

Appetite (%):	**Appetite Percentage of (Meals per day):**	**Snacks (Per day):**
80%	80%	

Appetite:

☐ Increase ☐ Decreased ☑ Unchanged

Intake:

☑ Adequate ☐ Inadequate

Nutritional Abnormalities:

☐ Anoxeria ☐ Cachexia ☐ Decreased Appetite ☐ Constipation

(K0520) Nutritional Approaches:

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	☐
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	☐
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	☐
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	☑
Z. None of the above	☐

#16 Nutritional Requirements (POC Element (§484.60 (2.ix))):

☐ NPO ☐ No Added Salt ☑ Low Salt ☐ Low Fat ☐ Low Cholesterol ☐ No Concentrated Sweets

☑ Controlled Carbohydrates ☐ Regular ☐ Fluid Restriction:

Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
◯ Without reason, has lost more than 10 lbs in the last 3 months	15	◯ Does not always have enough money to buy foods need	10
● Has an illness or condition that made pt change the type and/or amount of food eaten	10	◯ Eats alone most of the time	5
● Has open decubitus, ulcer, burn or wound	10		
◯ Eats fewer than 2 meals a day	10	● Takes 3 or more prescribed or OTC medications a day	5
◯ Has a tooth/mouth problem that makes it hard to eat	10	◯ Is not always physically able to cook and/or feed self and has no caregiver to assist	5
◯ Has 3 or more drinks of beer, liquor or wine almost every day	10	◯ Frequently has a diarrhea or constipation	5
		Total	25

◯ Good Nutritional Status (Score 0 - 25) ● Moderate Nutritional Status (Score 25 - 54) ◯ High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:
Instructed patient to follow therapeutic diet, including adequate fluid intake to ensure proper nutrition and hydration.

Weight:
☑ No Change ☐ Weight Gain ☐ Weight Loss
Comments:

(M1620) Bowel Incontinence Frequency:
● 0. Very rarely or never has bowel incontinence
◯ 1. Less than once weekly
◯ 2. One to three times weekly
◯ 3. Four to six times weekly
◯ 4. On a daily basis
◯ 5. More often than once daily
◯ NA - Patient has ostomy for bowel elimination
◯ UK - Unknown

(M1630) Ostomy for Bowel Elimination:
Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?
● 0. Patient does not have an ostomy for bowel elimination
◯ 1. Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.
◯ 2. The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.

Bowel Sounds (Abdominal Quadrants):

Right Lower:	Right Upper:	Left Lower:	Left Upper:	Last Bowel Movement:
Right Lower: Normoactive	Right Upper: Normoactive	Left Lower: Normoactive	Left Upper: Normoactive	04/11/2026

GI Symptoms:
☐ Nausea ☐ Vomiting ☐ Reflux/Indigestion ☐ Bowel Incontinence ☐ Diarrhea ☐ Constipation

Abdominal Assessment:
☑ Abdominal Soft/Non-tender ☐ Abdominal Soft/Tender ☐ Abdominal Distention ☐ Girth(cm):

Hemorrhoids:
☐ External Hemorrhoids ☐ Internal Hemorrhoids ☐ Laxative/Enema Use

Stool Appearance:
☐ Gray ☐ Tarry ☐ Brown/Tan ☐ Black ☐ Fresh Blood ☐ Other (Specify):

Ostomy/Stoma (if applicable):
☐ Moist ☐ Dry ☐ Pinkished-Red ☐ Purple ☐ Gray ☐ Black ☐ Odor

Peristomal Skin Characteristics:
☐ Clean ☐ Healthy ☐ Red ☐ Weeping ☐ Peeling ☐ Inflamed ☐ Other (Specify):
Comments:

Genitourinary (Genitals & Urinary)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?
● 0. No ◯ 1. Yes ◯ NA - Patient on prophylactic treatment ◯ UK - Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:
◯ Patient is incontinent
- ◯ Day time
- ◯ Night time
- ◯ Day and Night
- ◯ Timed-voiding defers incontinence
- ◯ Occasional stress incontinence

◯ Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)
● No incontinence or catheter (includes anuria or ostomy for urinary drainage)

Genitourinary Assessment:
☑ No Problems Identified

Abnormalities:
☐ Frequency ☐ Dysuria ☐ Urgency ☐ Anuria ☐ Nocturia ☐ Oliguria
☐ Retention ☐ Hematuria ☐ Irrigation ☐ Incontinence ☐ Diapers Used ☐ Penile Discharge
☐ Vaginal Bleeding/Discharge ☐ Other (Specify):

Urine Color:
☐ Transparent ☐ Pale Yellow ☐ Brownish Orange ☐ Pinkish Red

☑ Dark Yellow ☐ Blue or Green ☐ Foamy ☐ Transparent Yellow

Urine Consistency:

☑ Clear ☐ Sediment ☐ Odorous ☐ Cloudy

Urinary Catheter:

☐ Yes ☑ No

☐ External Catheter ☐ Foley ☐ Suprapubic ☐ Nephrostomy

☐ Last Catheter Change: mm/dd/yyyy ☐ Fr: ☐ cc:

Comments:

INTEGUMENTARY (SKIN & WOUND)

(M1306) Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?

(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

- ● 0. No **[→ Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC]**
- ○ 1. Yes

(M1311) Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage:

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non┐removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:

Intact skin with non blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persis tent blue or purple hues.

Stage 1 Pressure Injuries: **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:

(Excludes pressure ulcer/injury that cannot be staged due to a non removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)

Stage of Most Problematic: **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

- ○ 0. No **[GO TO M1340]**
- ○ 1. Yes, patient has BOTH observable and unobservable stasis ulcers
- ● 2. Yes, patient has observable stasis ulcers ONLY
- ○ 3. Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non┐removable dressing/device) **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

○ 1. One ○ 2. Two ● 3. Three ○ 4. Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable:

○ 1. Fully granulating ○ 2. Early/partial granulation ● 3. Not healing

(M1340) Does this patient have a Surgical Wound?

- ● 0. No
- ○ 1. Yes, patient has at least one observable surgical wound
- ○ 2. Surgical wound known but not observable due to non-removable dressing/device

(M1342) Status of Most Problematic Surgical Wound that is Observable:

○ 0. Newly epithelialized ○ 1. Fully granulating ○ 2. Early/partial granulation ○ 3. Not healing

Skin Color:

☑ Normal ☐ Pale ☐ Jaundice ☐ Cyanotic

Skin Turgor:

☐ Good ☑ Fair ☐ Poor

Skin Abnormalities:

☐ Dry ☐ Warm ☐ Cool ☐ Rash ☑ Ulcer ☐ Ostomy ☐ Incision ☐ Diaphoretic ☐ Other (Specify):

Comments:

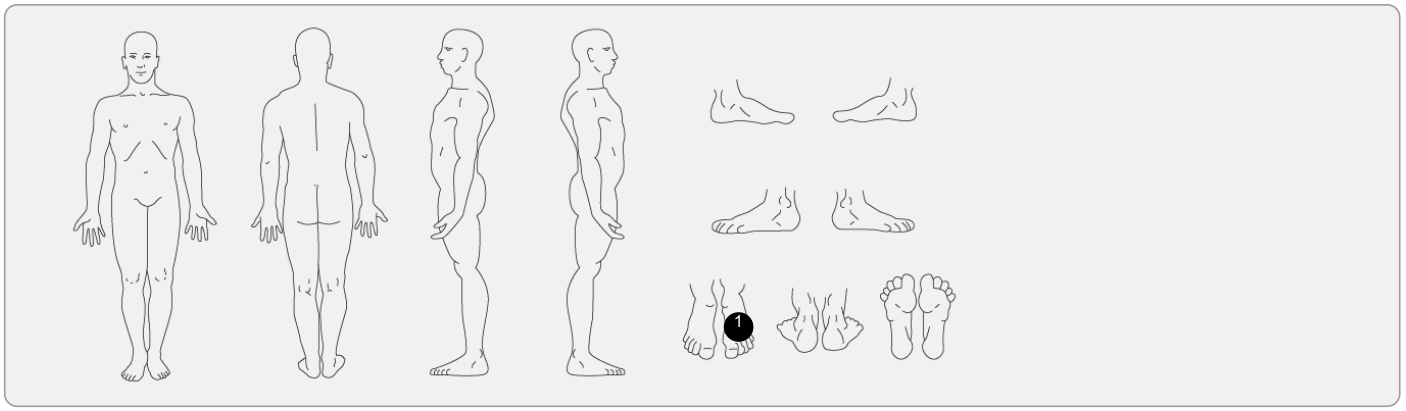
Norton Scale for Pressure Ulcer Risk Assessment:

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	○ Good	● Fair	○ Poor	○ Very Bad
Mental Condition	● Alert	○ Apathetic	○ Confused	○ Stuporous
Activity	○ Ambulant	● Walks With Help	○ Chairbound	○ Bedfast
Mobility	○ Full	● Slightly Impaired	○ Very Limited	○ Immobile
Incontinence	● None	○ Occassional	○ Usually Urinary	○ Urinary and Fecal
			Total Score:	17

Wound

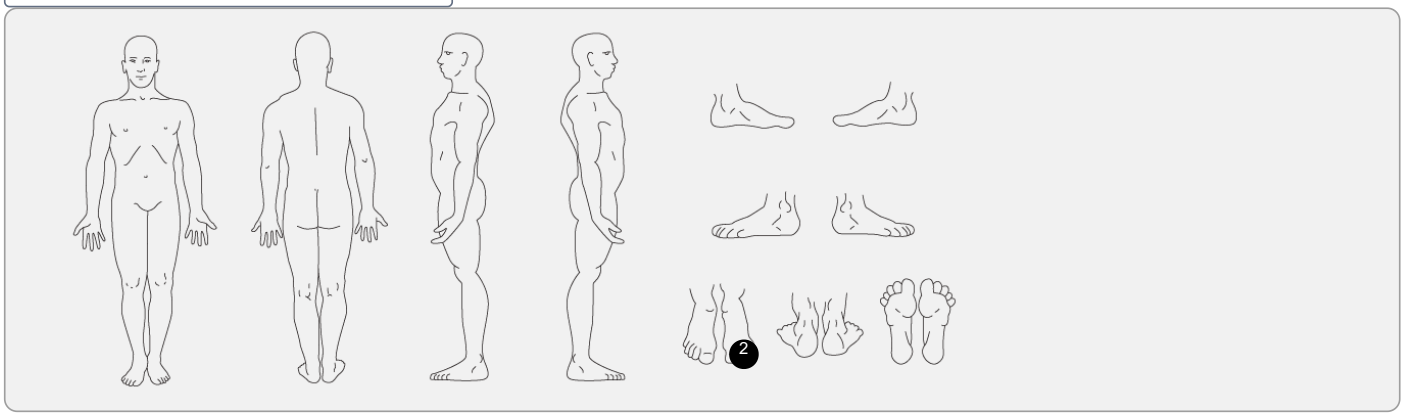
Wound Worksheet:

Fetch Wounds From Previous Wound Note



Wound 1

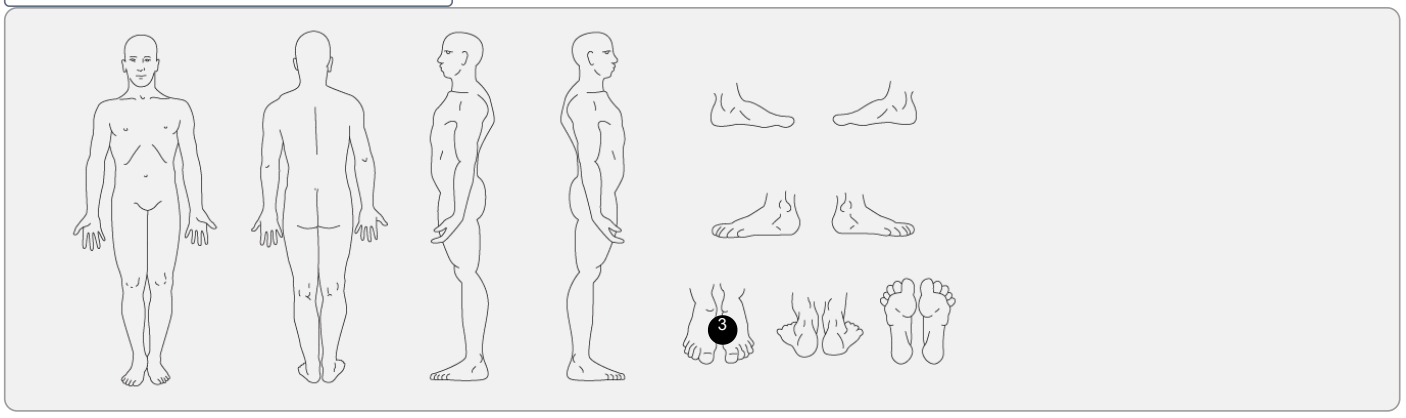
Location	Size	Pressure Injury Stage
dorsal aspect left foot	**4.0 x 3.0 x 0.0**	**-**
Type	Drainage	Amount Of Drainage
Venous Stasis Ulcer	**Serous**	**Minimal (<25% satur)**
Odor	Color	Tunneling/Undermining
Slight	**Yellow**	**-**
Tunneling Location	Tunneling Length (cm)	Undermining Location
-	**-**	**-**
Undermining Length (cm)	Surrounding Tissue	Wound Base
-	**-**	**-**
Level Of Pain	Edema	Stoma
5	**-**	**-**
Slough and/or Echar	Slough (%)	Eschar (%)
-	**-**	**-**
Granulation (%)	Pink/Red (%)	Necrotic (%)
-	**-**	**-**
Wound Status	Wound Description	Satisfaction of Return Demonstration
-	**oval shaped 4x3 cm superficial wound**	**- -**
Patient's Response to Wound Care	Wound Care	
Good - undefined	**Under aseptic technique, SN cleansed with wound cleanser, patted dry, applied xeroform, wrapped with kerlix, secured with tape. SN instructed PT/CG to perform ed wound dressing changed during non-SN visit days.**	
Onset Date		
-		



Wound 2

Location	Size	Pressure Injury Stage
2nd left toe	**0 x - x 0**	**-**
Type	Drainage	Amount Of Drainage
Venous Stasis Ulcer	**None**	**-**
Odor	Color	Tunneling/Undermining
Slight	**-**	**-**
Tunneling Location	Tunneling Length (cm)	Undermining Location
-	**-**	**-**

Undermining Length (cm)	Surrounding Tissue	Wound Base
-	-	-
Level Of Pain	**Edema**	**Stoma**
3	-	-
Slough and/or Echar	**Slough (%)**	**Eschar (%)**
-	-	-
Granulation (%)	**Pink/Red (%)**	**Necrotic (%)**
-	-	-
Wound Status	**Wound Description**	**Satisfaction of Return Demonstration**
-	dime size superficial wound	- -
Patient's Response to Wound Care	**Wound Care**	
Good - undefined	Under aseptic technique, SN cleansed with wound cleanser, patted dry, applied xeroform, wrapped with kerlix, secured with tape. SN instructed PT/CG to perform ed wound dressing changed during non-SN visit days.	
Onset Date		
-		



Wound 3

Location	Size	Pressure Injury Stage
medial aspect left ankle	0.5 x 0.8 x 0.0	-
Type	**Drainage**	**Amount Of Drainage**
Venous Stasis Ulcer	None	-
Odor	**Color**	**Tunneling/Undermining**
Slight	-	-
Tunneling Location	**Tunneling Length (cm)**	**Undermining Location**
-	-	-
Undermining Length (cm)	**Surrounding Tissue**	**Wound Base**
-	-	-
Level Of Pain	**Edema**	**Stoma**
3	-	-
Slough and/or Echar	**Slough (%)**	**Eschar (%)**
-	-	-
Granulation (%)	**Pink/Red (%)**	**Necrotic (%)**
-	-	-
Wound Status	**Wound Description**	**Satisfaction of Return Demonstration**
-	0.5x0.i8 cm superficial wound	- -
Patient's Response to Wound Care	**Wound Care**	
Good - undefined	Under aseptic technique, SN cleansed with wound cleanser, patted dry, applied xeroform, wrapped with kerlix, secured with tape. SN instructed PT/CG to perform ed wound dressing changed during non-SN visit days.	
Onset Date		
-		

SAFETY & RISK ASSESSMENT

Home Environment/Home Safety Assessment

- [ ] **Architectural Barriers**
- [ ] **No fire extinguisher**
- [ ] **No emergency exit plan**
- [ ] **Medications are improperly stored**
- [ ] **Inadequate electricity/cooling/heating**
- [ ] **Rodent/insect infestation**
- [ ] **No telephone**
- [ ] **Inadequate supply of water**
- [ ] **Unsafe electrical wiring**

- [ ] Cluttered surroundings
- [ ] Inadequate sanitation/plumbing
- [ ] Nonworking stove/refrigerator
- [ ] Inadequate railing
- [ ] Inadequate smoke detectors on all levels of home
- [ ] Exposed flammables
- [ ] Unsafe oxygen use
- [ ] Unsafe storage of equipments
- [ ] Inadequate Lighting

Others (Specify):

Type of Assistance Patient Receives - other than from Home Health Agency Staff:

ADL (Bathing, dressing, toileting, eating/feeding, bowel):
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances):
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

Psychosocial Support:
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

Assistance with doctors appointment, delivery of medications:
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

Management of Finances:
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

Comments:

(M1100) Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	○ Patient Lives alone around the clock	○ Patient Lives alone Regular daytime	○ Patient Lives alone Regular Night time	○ Patient Lives alone Short-term Assistance	○ Patient Lives alone No Assistance available
B. Patient lives with other person(s) in the home	● Patient lives with other person(s) in the home Around the clock	○ Patient lives with other person(s) in the home Regular daytime	○ Patient lives with other person(s) in the home Regular Night time	○ Patient lives with other person(s) in the home Short-term Assistance	○ Patient lives with other person(s) in the home No Assistance available
C. Patient lives in congregate situation (for example, assisted living, residential care home)	○ Patient lives in congregate situation (for example, assisted living, residential care home) around the clock	○ Regular daytime	○ Patient lives in congregate situation Regular Night time	○ Patient lives in congregate situation Short-term Assistance	○ Patient lives in congregate situation No Assistance available

(M2102) Types and Sources of Assistance

Determine the ability and willingness of non agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety *(for example, due to cognitive impairment)*

- ○ 0. No assistance needed – patient is independent or does not have needs in this area
- ○ 1. Non-agency caregiver(s) currently provide assistance
- ● 2. Non-agency caregiver(s) need training/supportive services to provide assistance
- ○ 3. Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance
- ○ 4. Assistance needed, but no non-agency caregiver(s) available

Safety Measures (POC Element (§484.60 (2.xi))):

- ● 1 - Bleeding precautions
- ○ 2 - O_2 precautions
- ○ 3 - Seizure precautions
- ● 4 - Fall precautions
- ○ 5 - Aspiration precautions
- ○ 6 - Siderails up
- ○ 7 - Elevate head of bed
- ○ 8 - 24 hr. supervision
- ● 9- Clear Pathways up
- ○ 10 - Lock w/c with transfers
- ● 11 - Infection control measures
- ● 12 - Walker/cane
- ○ 13 - Sharps Precaution
- ○ 14 - Other:

Hospitalization Risk Assessment

(M1033) Risk for Hospitalization:

Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- [x] 1. History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- [ ] 2. Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- [ ] 3. Multiple hospitalizations (2 or more) in the past 6 months
- [ ] 4. Multiple emergency department visits (2 or more) in the past 6 months
- [ ] 5. Decline in mental, emotional, or behavioral status in the past 3 months
- [ ] 6. Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- [x] 7. Currently taking 5 or more medications
- [ ] 8. Currently reports exhaustion
- [x] 9. Other risk(s) not listed in 1 - 8
- [ ] 10. None of the above

Additional Risk Information:
Patient has PAD noted with Venous Stasis Ulcer wounds, Cellulitis on left lower limb.

Fall Risk Test

- [ ] N/A
- [x] MAHC 10* - Fall Risk Assessment Tool
- [ ] TUG Assessment Tool

MAHC 10* - FALL RISK ASSESSMENT TOOL

Age 65+: ● Yes ○ No

Diagnosis (3 or more co-existing): ● Yes ○ No
- *Includes only documented medical diagnosis*

Prior history of falls within 3 months: ● Yes ○ No
- *An unintentional change in position resulting in coming to rest on the ground or at a lower level*

Incontinence: ○ Yes ● No
- *Inability to make it to the bathroom or commode in timely manner.*
- *Includes frequency, urgency, and/or nocturia.*

Visual impairment: ☑ Yes ☐ No

- *Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tol erance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.*

Impaired functional mobility: ☑ Yes ☐ No

- *May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, imp aired coordination or improper use of assistive devices.*

Environmental hazards: ☐ Yes ☑ No

- *May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.*

Poly Pharmacy (4 or more prescriptions-any type): ☑ Yes ☐ No

- *All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquili zers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.*

Pain affecting level of function: ☑ Yes ☐ No

- *Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.*

Cognitive impairment: ☐ Yes ☑ No

- *Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsi vity, memory deficits. Consider patients ability to adhere to the plan of care.*

A score of 4 or more is considered at risk for falling: 6.00

Fire Assessment for Patient with Oxygen

☑ **Patient is not using oxygen**

FUNCTIONAL ASSESSMENT (SELF CARE)

(GG0100) Prior Functioning: Everyday Activities:

Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.

- ***3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper*
- ***2. Needed Some Help** – Patient needed partial assistance from another person to complete activities*
- ***1. Dependent** – A helper completed the activities for the patient.*
- ***8. Unknown***
- ***9. Not Applicable***

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	○	✓	○	○	○
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	○	✓	○	○	○
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	○	✓	○	○	○
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	○	✓	○	○	○

(GG0110) Prior Device Use:

*Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (**Check all that apply**)*

- ☐ **A. Manual wheelchair**
- ☐ **B. Motorized wheelchair and/or scooter**
- ☐ **C. Mechanical lift**
- ☑ **D. Walker**
- ☐ **E. Orthotics/Prosthetics**
- ☐ **Z. None of the above**

(GG0130) Self Care

(GG0130) SOC/ROC Performance:

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.

Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.

Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance –
Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:

07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0130A) A. Eating:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ☑ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130B) B. Oral Hygiene:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ☑ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130C) C. Toileting Hygiene:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ☑ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130E) E. Shower/Bathe Self:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ☑ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130F) F. Upper Body Dressing:

(GG0130G) G. Lower Body Dressing:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130H) H. Putting on/Taking Off Footwear:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(M1800) Grooming:

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- ○ 0. Able to groom self unaided, with or without the use of assistive devices or adapted methods.
- ○ 1. Grooming utensils must be placed within reach before able to complete grooming activities.
- ● 2. Someone must assist the patient to groom self.
- ○ 3. Patient depends entirely upon someone else for grooming needs

(M1810) Ability to Dress Upper Body:

*Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:*

- ○ 0. Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.
- ○ 1. Able to dress upper body without assistance if clothing is laid out or handed to the patient.
- ● 2. Someone must help the patient put on upper body clothing.
- ○ 3. Patient depends entirely upon another person to dress the upper body.

(M1820) Ability to Dress Lower Body:

*Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:*

- ○ 0. Able to obtain, put on, and remove clothing and shoes without assistance.
- ○ 1. Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.
- ● 2. Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.
- ○ 3. Patient depends entirely upon another person to dress lower body.

(M1830) Bathing:

*Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).***

- ○ 0. Able to bathe self in shower or tub independently, including getting in and out of tub/shower.
- ○ 1. With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.
- ○ 2. Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.
- ● 3. Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.
- ○ 4. Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.
- ○ 5. Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person
- ○ 6. Unable to participate effectively in bathing and is bathed totally by another person.

(M1840) Toilet Transferring:

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- ○ 0. Able to get to and from the toilet and transfer independently with or without a device
- ○ 1. When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.
- ● 2. Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).
- ○ 3. Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently
- ○ 4. Is totally dependent in toileting.

(M1845) Toileting Hygiene:

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ○ 0. Able to manage toileting hygiene and clothing management without assistance.
- ○ 1. Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.
- ● 2. Someone must help the patient to maintain toileting hygiene and/or adjust clothing.
- ○ 3. Patient depends entirely upon another person to maintain toileting hygiene

(M1850) Transferring:

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ○ 0. Able to independently transfer.
- ○ 1. Able to transfer with minimal human assistance or with use of an assistive device.
- ● 2. Able to bear weight and pivot during the transfer process but unable to transfer self.
- ○ 3. Unable to transfer self and is unable to bear weight or pivot when transferred by another person
- ○ 4. Bedfast, unable to transfer but is able to turn and position self in bed.
- ○ 5. Bedfast, unable to transfer and is unable to turn and position self

(M1860) Ambulation/Locomotion:

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ○ 0. Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).
- ○ 1. With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.
- ○ 2. Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.
- ● 3. Able to walk only with the supervision or assistance of another person at all times.
- ○ 4. Chairfast, unable to ambulate but is able to wheel self independently.
- ○ 5. Chairfast, unable to ambulate and is unable to wheel self
- ○ 6. Bedfast, unable to ambulate or be up in a chair.

(GG0170A) A. Roll left and right:

The ability to roll from lying on back to left and right side, and return to lying on back on the bed.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170B) B. Sit to lying:

The ability to move from sitting on side of bed to lying flat on the bed.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170C) C. Lying to sitting on side of bed:

The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170D) D. Sit to stand:

The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170E) E. Chair/bed-to-chair transfer:

The ability to transfer to and from a bed to a chair (or wheelchair).

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170F) F. Toilet transfer:

The ability to get on and off a toilet or commode.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170G) G. Car transfer:

The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170I) I. Walk 10 feet:

Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170J) J. Walk 50 feet with two turns:

Once standing, the ability to walk 50 feet and make two turns.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170K) K. Walk 150 feet:

Once standing, the ability to walk at least 150 feet in a corridor or similar space.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170L) L. Walking 10 feet on uneven surfaces:

The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

- ◯ 01. Dependent

(GG0170M) M. 1 step (curb):

The ability to go up and down a curb and/or up and down one step. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ☑ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170N) N. 4 steps:

The ability to go up and down four steps with or without a rail. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170O) O. 12 steps:

The ability to go up and down 12 steps with or without a rail.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170P) P. Picking up object:

The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ☑ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170Q) Q. Does patient use a wheelchair and/or scooter?

☑ 0. No ◯ 1. Yes ◯ (-) Not assessed/no information

(GG0170R) R. Wheel 50 feet with two turns:

Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170RR) RR1. Indicate the type of wheelchair or scooter used.

◯ 1. Manual ◯ 2. Motorized ◯ (-) Not assessed/no information

(GG0170S) S. Wheel 150 feet:

Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170SS) SS1. Indicate the type of wheelchair or scooter used.

◯ 1. Manual ◯ 2. Motorized ◯ (-) Not assessed/no information

Gait and Mobility Abnormalities:

☐ No Problems Identified

☑ Weakness	☑ Poor Balance	☐ Bed Bound	☐ Chair Bound
☐ Joint Stiffness	☐ Shuffling/Wide-based Gait	☐ Orthopedic Aftercare	☑ High Risk for Falls
☐ Weight Bearing Restriction		☑ Poor Conditioning	
☐ Fracture:		☐ Joints Pain:	
☐ Amputation:		☐ Atrophy:	
☐ Paresthesia:		☐ Contractures (Site):	
☐ Foot Drop:	☐ Right ☐ Left		

Fall(s) since last visit (Specify):

Weakness (Specify):

Change in ADL (Explain):

Comments:

Range of Motion:

☑ Limited Range of Motion: ☐ RUE ☐ RLE ☐ LUE ☑ LLE ☐ Quadriplegia ☐ Hemiplegia ☐ Paraplegia

Comments:

Grip Strength:

☑ Normal ☐ Strong ☐ Weak ☑ Equal ☐ Unequal ☐ Bilateral ☐ Right ☐ Left

Others (Specify):

Comments:

DME and Supplies (POC Element (§484.60 (2.iii))):

DME Lookup

Durable Medical Equipment:
Walker

Wound:
4x4's, Cotton tipped applicators, Wound cleanser, Non-sterile Gloves, Kerlix, Tape

#18B Activities Permitted (POC Element (§484.60 (2.viii))):

- ○ 1- Complete bedrest
- ○ 2 - Bedrest/BRP
- ☑ 3 - Up as tolerated
- ☑ 4 - Transfer bed/chair
- ☑ 5 - Exercises prescribed
- ○ 6 - Partial weight bearing
- ○ 7 - Independent in home
- ○ 8 - Crutches
- ○ 9 - Cane
- ○ A - Wheelchair
- ☑ B - Walker
- ○ C - No restrictions
- ○ D - Other (specify)

Rehabilitation Potential (§484.60 (2.vi)):

- ○ Poor
- ☑ Fair
- ○ Good
- ○ Excellent

Others

ENDOCRINE (DIABETIC MANAGEMENT)

Endocrine

Is patient diabetic? ☐ Yes ☑ No **If Yes, How Long?**

Diabetes Type:
☐ Insulin-Dependent Diabetes Mellitus (IDD/Type 1) ☐ Non-Insulin-Dependent Diabetes Mellitus (NIDDM/Type 2)

Insulin Administered By:
☐ Patient ☐ Caregivier ☐ SN

Glucose Level Issue:
☐ Hypoglycemia (Low Blood Sugar) ☐ Hyperglycemia (High Blood Sugar)

Frequency of Blood Sugar Issues:
☐ Daily ☐ Weekly ☐ Monthly ☐ Rarely ☐ As needed

Does patient or caregiver routinely perform inspection of the patient's lower extremities?
☐ Yes ☐ No ☐ N/A

Symptoms & Observations:

- ☐ Fatigue (related to blood sugar)
- ☐ Shakiness/Tremors
- ☐ Sweating
- ☐ Confusion/Irritability
- ☐ Hunger
- ☐ Dizziness
- ☐ Blurred Vision
- ☐ Dry Mouth/Excessive Thirst
- ☐ Frequent Urination
- ☐ Weight Loss (unintentional)
- ☐ Weight Gain (unintentional)

Specify Other Symptoms or Patterns:

PLAN OF CARE ORDERS AND DISCHARGE PLAN

Plan of Care Orders

(POC Element):

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 1x/wk x 1 wk, 2x/wk x 4 wks

MSW Frequency:

PT Frequency: evaluation and treatment

OT Frequency:

HHA Frequency: 2x/wk x 2wks, 1x/wk x 2 wks effective on week of 04/12/26

ST Frequency

Dietitian Frequency:

*****MD SIGNATURE COVERS ALL PAGES*****
ADMIT PATIENT TO A PLUS HOME HEALTH SYSTEMS LLC SERVICES. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

James Helms, a 90-year-old male, is being referred for home health care following an encounter for change or removal of a non-surgical wound dressing, with a chronic ulcer on another part of the left foot with the fat layer exposed; he recently underwent a revascularization procedure due to significant Peripheral Artery Disease and has a left second toe ulceration at risk for infection requiring skilled wound care, with additional conditions including cellulitis of the left foot and a blister on the left foot, and following vascular surgery he continues to experience pain and significant swelling in his lower extremities, indicating compromised circulation and the need for ongoing monitoring and skilled nursing care.

Frequency and duration:
Skilled Nurse Visit Frequency: 1 time a week for 1 week, 2 times a week for 4 weeks for skilled observation and assessment, medication review and reconciliation, pain management, wound assessment and wound care infection control measures, compliance with treatment regimen and therapeutic diet, home safety and disease process teaching.
Physical Therapy Visit Frequency: Evaluation and treatment for mobility, gait, transfer, established therapeutic exercises, strengthening, home safety and fall prevention techniques training.
HHA Visit Frequency: 2 times per week for 2 weeks, 1 time a week for 2 weeks for ADLs/personal care assistance effective on week of 04/12/26.

Wound Care Order:
Wound #1: Venous Stasis Ulcer on dorsal aspect left foot under aseptic technique, SN to cleanse with wound cleanser, pat dry, apply xeroform, wrap with kerlix, secure with tape. Change dressing every 2 to 3x weekly and as needed when soiled or dislodged. SN to instruct PT/CG to perform wound dressing change during non-SN visit days.
Wound #2: Venous Stasis Ulcer on 2nd left toe under aseptic technique, SN to cleanse with wound cleanser, pat dry, apply xeroform, wrap with kerlix, secure with tape. Change dressing every 2 to 3x weekly and as needed when soiled or dislodged. SN to instruct PT/CG to perform wound dressing change during non-SN visit days.
Wound #3: Venous Stasis Ulcer on medial aspect left ankle under aseptic technique, SN to cleanse with wound cleanser, pat dry, apply xeroform, wrap with kerlix, secure with tape. Change dressing every 2 to 3x weekly and as needed when soiled or dislodged. SN to instruct PT/CG to perform wound dressing change during non-SN visit days.

Patient goals: The Patient aims to improve overall health, functional ability, independence, and understanding of self-care management. Patient also wishes wound to heal without any infection and be free from pain. edema and signs and symptoms of disease exacerbation, as well as avoid falls and injuries.

Skilled Intervention

Assessment/Instruction/Performance:
Patient Covid 19 screening completed prior to visit. No report of symptoms or known COVID-19 exposure per patient/family. Patient provided with mask for use during HH visit. Patient agreed to use mask during treatment. Clinician performed hand hygiene, donned mask, face shield and gloves and followed infection control and bag technique procedures during the visit. No COVID related concerns during the time of visit. Disinfection of equipment followed including dry time. Proper disposal and doffing is done upon completion of the visit.

Explained Home Health service to patient/family/caregiver(s). Explained consent to care, release of liability, and patient rights & responsibilities. Consent to care, a liability release, and consent to photograph were signed by the patient/responsible party. A comprehensive assessment of all body systems with OASIS elements evaluation was initiated. Reconciled facility discharge medications against routine home prescriptions without issue. Discussed advance directives, privacy policy, COVID-19 prevention, 911 protocols, basic home safety, and fall prevention principles with patient/family/caregiver(s). Discussed the plan of care draft with the patient and explained what to expect in the next visits and the remainder of the care episode. Also explained that the initial

discussion on the management of other disease processes will be done on the next visit.

SN assessed the patient's venous stasis ulcers located on the dorsal aspect of the left foot, 2nd left toe, and medial aspect of the left ankle, noting wound condition, drainage, and surrounding skin for any signs of infection or deterioration. SN performed wound care using aseptic technique, cleansed the areas with wound cleanser, gently patted them dry, applied xeroform to maintain a moist healing environment, and wrapped with kerlix secured with tape. SN actively monitored for redness, swelling, odor, or increased drainage and reinforced the importance of proper circulation, including elevating the affected limb when possible. SN taught the patient/caregiver on proper wound care techniques, infection prevention, and signs to report promptly. Dressing changes were completed as ordered, to be done 2–3 times weekly and as needed if soiled or dislodged. SN also instructed the patient/caregiver to perform wound care during non-visit days and ensured understanding through return demonstration and verbal feedback.

SN assessed the patient with +2 pitting edema on bilateral lower extremities and cellulitis on the left lower extremity, noting increased swelling, skin warmth, redness, and tenderness. SN monitored for any signs of worsening condition such as increased pain, spreading redness, or fever. SN elevated the lower extremities to help reduce swelling and promote venous return and encouraged the patient to avoid prolonged standing or sitting without movement. SN reinforced the importance of proper skin hygiene and keeping the affected area clean and dry. SN also instructed on measures to reduce edema, including leg elevation, gentle range of motion, and use of compression as tolerated and as ordered. SN taught the patient/caregiver on recognizing early signs of complications and when to report changes to the healthcare provider and ensured understanding through verbal feedback.

SN assessed non-productive cough (frequency, duration, associated symptoms) and evaluated lung sounds/respiratory status. Encouraged fluids and humidified air, proper upright positioning, and avoidance of irritants. Reinforced adherence to prescribed treatments. Instructed patient/caregiver to monitor and report worsening symptoms (productive cough, fever, dyspnea); understanding confirmed. SN assessed bleeding precautions related to anticoagulant use, monitoring for signs of bleeding (bruising, petechiae, gum/nose bleeding, hematuria, blood in stool) and checked vital signs for instability. Reinforced safety measures including soft toothbrush, electric razor, fall/injury prevention, and avoiding sharp objects and alcohol. Instructed medication adherence without dose changes. Taught patient/caregiver on warning signs and when to seek urgent care; understanding confirmed.

SN assessed the patient reporting 5/10 pain on the dorsal aspect of the left foot, noting location, intensity, and any factors that aggravated or relieved the discomfort. SN assessed for associated symptoms such as redness, swelling, or limited mobility. SN encouraged the patient to elevate the affected extremity and avoid excessive pressure or prolonged standing to help reduce discomfort. SN reinforced non-pharmacological pain management techniques such as rest, proper positioning, and use of supportive footwear. SN also instructed the patient to follow prescribed pain management measures as directed and to report any increase in pain or new symptoms. SN taught the patient/caregiver to monitor pain levels regularly and to notify the healthcare provider if pain worsens or becomes unrelieved, ensuring understanding through verbal feedback.

SN assessed patient with generalized muscle weakness and high fall risk, evaluating gait, balance, and mobility status. SN ensured fall precautions in place, including safe environment, clutter-free surroundings, proper lighting, and assistive device use as needed. SN reinforced need for assistance with ambulation and transfers. SN encouraged energy conservation and gradual activity as tolerated. SN instructed patient/caregiver on fall prevention strategies and to report any dizziness, weakness, or near-fall incidents; understanding confirmed via verbal feedback.

Re-check pain.

Verbalize understanding.

Conference With:

☑ **MD** ☐ **SN** ☑ **PT** ☐ **OT** ☐ **ST** ☐ **MSW** ☑ **HHA** ☐ **DON/DOPCS** ☑ **Clinical Coordinator** ☑ **Clinical Manager**

Name:
Kristine Macaraeg -RN CM

Regarding:
SOC, POC, Visit frequencies and update of patient health condition, wound care consult, shower assist, PT eval

Physician Contacted Re:
SOC, POC, Visit frequencies and update of patient health condition, wound care consult, shower assist, PT eval

Order Changes:

Plan for Next Visit:
SN will continue to assess the patient's overall health condition, wound assessment and wound care patient/PCG understanding of the disease process, functional status, and knowledge of self-care management by the next visit.

Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge (POC Element (§484.60 (2.xiii))):

Ongoing education and training provided by the SN, as appropriate, regarding the care and services identified in the plan of care.

Discharge Plans

(POC Element):

Able to understand medication regime and care related to diagnoses.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Discharge patient to self care.
Discharge to caregiver.
Discharge when goals met. Discharge when wound(s) healed.
When goals are met.

☐ **Written notice of discharge provided to patient**

Discharge scheduled for:
mm/dd/yyyy

Discharge Plan Discussed with:

☑ **Patient/Family** ☐ **Care Manager** ☐ **Physician** ☐ **Other (specify):**

Discharge Plan:

☑ **To self-care when goals met** ☐ **To self care when max potential achieved** ☐ **To outpatient therapy with MD approval**

☐ **Others (Specify):**

Comments:

PATIENT SUMMARY & CLINICAL NARRATIVE

Summary

(POC Element):

James Helms, a 90-year-old male, seen for wound care of a chronic toe ulcer and a new, recurrent blister on the dorsal left foot with drainage. His history includes an artificial heart valve, atrial fibrillation, recent left leg vascular surgery with stent, and anticoagulation with Warfarin and aspirin. Assessment revealed multiple non-healing wounds, elevated glucose, and pressure points from footwear. Plan included disconstricting sandals, wound care, PT for fall risk, home health for showering, and smoking cessation counseling.
Chief Complaints
Toe pain and blister on top of foot that comes and goes and leaks

History of Present Illness
James Helms is a 90-year-old male with a history of artificial heart valve replacement, recent vascular surgery with stent placement, and anticoagulation therapy who presents for wound care management of chronic foot ulcerations and blistering.

The patient has been experiencing issues with his toe for an extended period, with the caregiver noting this has been the "main thing." Additionally, he has developed a blister on the top of his foot that intermittently appears, disappears, and leaks fluid. The toe issue has been present for a long time, while the foot blister developed more recently, with the patient being taken to a podiatrist on March 27th after the blister started small and progressively enlarged over a couple of days.

The patient reports pain in the toe area and describes experiencing pain "a little bit" in various locations on his foot, including between toes, where the blister is located, and on the back of his foot. He also mentions that his "little one has been bothering me lately," referring to his little toe. The pain is described as intermittent, with the patient stating "I get pain" but noting it's "not there now" during the examination.

Recent healthcare interactions include an urgent care visit where a small sore on his toe was identified. Following this, he had a follow-up appointment with his primary care provider while his caregiver was out of town. The primary care provider diagnosed him with "a raging case of athlete's foot" and prescribed antifungal medication without physically examining his feet, according to the caregiver. When his foot subsequently swelled significantly, he was given diuretics. A subsequent podiatrist evaluation contradicted the athlete's foot diagnosis and led to vascular surgery consultation and intervention.

The patient underwent vascular surgery on March 31st for arterial revascularization with stent placement in his left leg, with plans to discuss intervention for the other leg at an upcoming follow-up appointment. He is currently on anticoagulation therapy with Coumadin, with INR monitoring every two weeks, and his most recent INR was 2.4 as of Monday.

The patient's current footwear consists of sandals that were purchased when he was initially diagnosed with athlete's foot, though he typically wears socks and tennis shoes. His caregiver has been managing wound care at home using petroleum-based products, gauze, and tape as directed, with dressing changes performed most recently on Thursday night.

Medical History
- Artificial heart valve replacement
- Atrial fibrillation
- Peripheral arterial disease with angioplasty and stent placement to left leg on March 31, 2026
- History of falls
- Memory issues described as "hit and miss"

Surgical History
- Heart valve replacement with metal valve (artificial valve)
- Angioplasty and stent placement in left leg on March 31st

Medications and Supplements
- Warfarin (Coumadin) 2.5 mg and 5 mg by mouth
- Takes 5 mg four days a week and 2.5 mg three days a week
- Followed by Dr. Andrew Ayers with labs every 2 weeks
- Most recent INR was 2.4 as of Monday
- Baby aspirin by mouth daily
- Started after surgery
- Antifungal medication
- Given by primary care for presumed athlete's foot

Allergies
- No known allergies to medications

Social History
- Substance Use: Current smoker (1 pack per day)
- Living Situation: Lives at home with daughter who provides care and assistance
- Occupation: Retired Air Force veteran
- Social Support: Daughter serves as primary caregiver and accompanies to medical appointments

Review of Systems
General: Positive for lightheadedness.
Cardiovascular: Negative for chest pain.
Neurological: Positive for hit and miss memory issues.
Vital Signs
- Blood Pressure: 120/62 mmHg

Physical Examination
General: 90-year-old male who appears stated age. Uses walker for ambulation.
Respiratory: Lung sounds clear bilaterally.
Skin: Multiple foot lesions present. Large blister on dorsal aspect of left foot that comes and goes and leaks fluid. Chronic sore on toe. Additional sore on medial aspect of foot. Sore on posterior ankle causing ankle pain. Skin breakdown noted at pressure points from sandal straps. Tan lines visible on feet consistent with sandal wear pattern.
Musculoskeletal: Ankle range of motion intact with dorsiflexion and plantarflexion. Foot flexibility maintained.
Neurological: Oriented to person, place, and time. Reports memory as "hit and miss." Feet noted to be sensitive to touch with areas of both pain and tickling sensation during examination.

Laboratory, Imaging, and Diagnostic Test Results
- INR: 2.4 (as of Monday, April 6, 2026)
James Helms is a 90-year-old male with history of artificial heart valve, recent vascular surgery, and anticoagulation therapy presenting for home health evaluation of chronic toe ulcer and new foot blistering with concerns for poor wound healing.

Chronic toe ulcer with new foot blistering
Assessment: Patient has a longstanding ulcer on his toe that has been present since at least February 21st, initially misdiagnosed and treated as athlete's foot by primary care. New blistering has developed on the dorsal aspect of his foot that comes and goes with drainage. Multiple pressure points and skin breakdown are noted on both feet, including sores on the inside of the foot and back of ankle. The constellation of multiple non-healing wounds, elevated blood glucose levels, and poor circulation from smoking raises significant concern for diabetic ulceration, though diabetes has not been formally diagnosed. Current footwear (tight sandals) is contributing to pressure points and preventing healing.
Plan:
- Discontinue current sandals due to constricting pressure on wounds
- Provide non-slip socks for indoor use instead of shoes
- Request wound care consultation for specialized assessment and management recommendations
- Coordinate with existing podiatrist care (patient has follow-up scheduled for April 21st)
- Obtain hemoglobin A1C to evaluate for diabetes given elevated glucose and poor wound healing
- Frequent dressing changes as needed
- Patient education on proper foot care and importance of appropriate footwear

Fall risk and mobility issues
Assessment: Patient has had recent falls, including one "the other day" when going to the car where he caught himself. He uses a walker but admits to being "stubborn" and not consistently using it. Current walker has brakes which may actually increase fall risk if patient cannot engage them quickly enough. Patient reports feeling lightheaded and having muscle weakness, particularly concerning given his anticoagulation status and risk for serious injury from falls.
Plan:
- Order physical therapy evaluation for strengthening and balance assessment
- Patient education on consistent walker use, especially given anticoagulation risk

Poor hygiene and shower safety
Assessment: Patient showers infrequently (approximately once per week) and requires assistance due to mobility limitations and fall risk. Proper hygiene is critical for wound healing, and patient experiences lightheadedness and difficulty with lower leg care during showering. Hot water can cause vasodilation leading to increased dizziness and fall risk.
Plan:
- Arrange home health aide for shower assistance 2-3 times per week until wound healing improves
- Use shower chair consistently for safety
- Keep affected foot dry during showering with protective covering
- Patient education on importance of hygiene for wound healing

Anticoagulation management
Assessment: Patient is on warfarin therapy following recent artificial valve replacement and vascular surgery on March 31st. Current regimen is 5mg four days per week and 2.5mg three days per week. Most recent INR was 2.4 as of Monday, which is within therapeutic range. INR monitoring is being done every two weeks with full blood work.
Plan:
- Continue INR monitoring every two weeks
- Follow-up with Dr. Andrew Ayers for anticoagulation management

Tobacco use
Assessment: Patient smokes one pack per day, which significantly impairs circulation and wound healing by constricting blood vessels in the extremities. This is particularly concerning given his current foot wounds and vascular disease.
Plan:
- Patient education on smoking's impact on wound healing and circulation
Wound Care:
- Stop wearing sandals immediately - they are too tight and causing pressure sores
- Wear non-slip socks instead of shoes when possible
- Use loose, non-slip slippers for walking around the house
- For going out, wear very loose regular socks and loose sneakers only when necessary

- Keep foot dry during showering - do not soak the wound
- Use a shower chair when bathing
- Leave the foam cushion on your foot to protect the pressure point

Medications:
- Continue taking all as prescribed

Safety:
- Use your walker consistently to prevent falls
- Accept help with showering until foot heals
- Have someone present when showering due to dizziness risk

Follow-up Appointments:
- Podiatrist appointment on April 21st
- Vascular follow-up next week
- INR lab work needed today

Referrals:
- Physical therapy evaluation for balance and strength
- Home health aide for shower assistance
- Wound care consultation

Lifestyle:
- smoking cessation counseling done

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Chronic Wound (e.g., Diabetic Ulcer, Pressure Ulcer)
Goals: Target Completion: 60 1 Term: Long-term Status: Unmet
Promote healing of the chronic wound.
Interventions: Assigned Staff: HHA, SN
Apply appropriate wound dressings and monitor for signs of infection.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on wound care techniques.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 60 1 Term: Long-term Status: Unmet
Prevent recurrence of the chronic wound.
Interventions: Assigned Staff: HHA, SN
Develop a skin care regimen to maintain skin integrity.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 60 1 Term: Long-term Status: Unmet
Reduce the risk of falls in the home environment.
Interventions: Assigned Staff: OT, SN
Conduct a home safety assessment and recommend modifications.
Progress Toward Goals:
No Progress Yet
Implement strength and balance exercises to improve stability.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 60 1 Term: Long-term Status: Unmet
Enhance mobility and independence.
Interventions: Assigned Staff: PT, SN
Engage in prescribed physical therapy for mobility improvement.
Progress Toward Goals:
No Progress Yet

Problem: Hypertension
Goals: Target Completion: 60 1 Term: Long-term Status: Unmet
Achieve and maintain target blood pressure levels.
Interventions: Assigned Staff: SN
Regular monitoring of blood pressure at home.
Progress Toward Goals:
No Progress Yet

Problem: Chronic Wound (Diabetic Ulcer)
Goals: Target Completion: 60 1 Term: Long-term Status: Unmet
Promote healing of the diabetic ulcer.
Interventions: Assigned Staff: HHA, SN
Perform wound care daily using appropriate techniques.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on diabetes management and its impact on wound healing.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 60 1 Term: Long-term Status: Unmet
Reduce the risk of falls.
Interventions: Assigned Staff: SN
Conduct a comprehensive fall risk assessment.
Progress Toward Goals:
No Progress Yet
Implement a home safety evaluation and make necessary modifications.
Progress Toward Goals:
No Progress Yet

Problem: Hypertension
Goals: Target Completion: 60 1 Term: Long-term Status: Unmet
Maintain blood pressure within normal range.
Interventions: Assigned Staff: SN
Monitor blood pressure regularly and document findings.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (04/11/2026):

Assigned Staff: Miller, Julie RN

Electronically Signed by Miller, Julie RN 04/11/2026